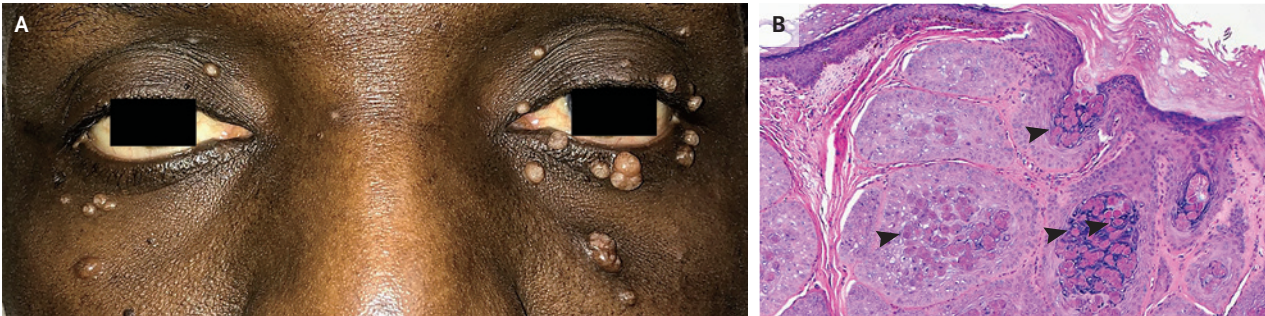


IMAGES IN CLINICAL MEDICINE

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Molluscum Contagiosum



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A 60-YEAR-OLD MAN WITH A HISTORY OF HUMAN IMMUNODEFICIENCY virus (HIV) infection presented to the ophthalmology clinic with lesions on his eyelids and face that had increased in number during the preceding 4 months. His CD4 cell count was 20 per cubic millimeter (reference range, 500 to 1500) and his HIV viral load was 120,000 copies per milliliter. Numerous dome-shaped, flesh-colored papules with central umbilication were observed on both the eyelids and the cheeks (Panel A). Incision and curettage of all the lesions were performed, followed by the application of prophylactic antibiotic ointment. Histopathological assessment of the lesions showed coarse, basophilic, keratohyaline granules with large intracytoplasmic, eosinophilic inclusion bodies (Panel B, arrowheads), confirming a diagnosis of molluscum contagiosum. Molluscum contagiosum is caused by a poxvirus and leads to chronic, localized infection of the skin that can occur anywhere on the body. Patients with cellular immunodeficiency are at risk for more severe, treatment-resistant infection. The differential diagnosis for molluscum contagiosum includes cryptococcosis, histoplasmosis, basal-cell carcinoma, and warts. The patient was referred for HIV treatment. At a follow-up visit 6 months later, he had had no recurrence of the lesions.

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